Aster LABS
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MC-3362

TEST REPORT

Lab. Id	: P2050512	**Hosp. UHID** :		**Reg. Date**	: 27-Apr-2024 / 12:02 PM
Name	: MR. MANAK SAHU .			**Collection**	: 27-Apr-2024 / 09:30 AM
Age/Gender	: 36Y-1D / Male			**Received**	: 27-Apr-2024 / 15:26 PM
Collected At	: ASTER CLINIC (MANYATA)			**Report**	: 27-Apr-2024 / 16:53 PM
Referral Dr	: DR WELLNESS.	**Report Status :** Final		**Print**	: 27-Apr-2024 / 18:41 PM
Bed	:				

Haematology

Investigation	Observed Values	units	Biological Refer Range	Specimen

BLOOD PICTURE PERIPHERAL BLOOD SMEAR EXAMINATION

Specimen Type	EDTA Whole Blood
Method	Manual,Microscopy
PBS No :	1780/24
Adequacy :	Adequate
RBC Morphology :	Normocytic hypochromic.Few Microcytic Hypochromic RBCs seen. No anisopoikilocytosis seen. No fragmented RBCs, No NRBCs seen.
WBC Morphology :	WBCs are normal in number, morphology and distribution.
Platelet Morphology :	Adequate in number with normal morphology.
Additional findings :	No malarial parasite seen.
Impression	Normocytic Normochromic Anemia.

Hema N Anadure

Verified By: 171886

Dr. HEMA N ANADURE
M.D (Pathology)
Consultant

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Biochemistry

Parameter	Results	Units	Biological Ref Interval	Specimen
BLOOD UREA NITROGEN Method:UREASE	9.35	mg/dl	7-18	Serum
UREA Method:UREASE	20	mg/dl	< 39	Serum

Comments:
Conditions that can increase BUN levels include:
1.Kidney disease, kidney damage, or kidney failure
2.Decreased blood flow to the kidneys, caused by conditions such as congestive heart failure, shock, stress, recent heart attack, or severe burns
3.Conditions that can block the flow of urine, such as kidney stones
4.Dehydration
5.Increased protein breakdown or significantly increased protein in the diet
Decreased BUN may be seen in severe liver disease, malnutrition, and sometimes when you are overhydrated (too much fluid volume), but the BUN test is not usually used to diagnose or monitor these conditions.

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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Haematology

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
HAEMOGRAM (CBC+ESR)				
RBC Parameters				
Haemoglobin Method:Photometry	**10.5**	gm/dl	13.0-17.0 g/dL	K2EDTA WB
RBC Count Method:Electrical Impedance/Flowcytometry	**4.4**	mill/ul	4.5-5.5 mil/cmm	K2EDTA WB
PCV Method:Calculated	**35.7**	%	40-51	K2EDTA WB
MCV Method:Calculated	80.5	fl	80-99	K2EDTA WB
MCH Method:Calculated	**23.7**	pg	27.0-33.5	K2EDTA WB
MCHC Method:Calculated	**29.4**	g/dL	31.5-36.0	K2EDTA WB
RDW-CV Method:Calculated	**18.8**	%	11.6-14.0	K2EDTA WB
WBC Parameters				
Total WBC Count Method:Electrical Impedance/Flowcytometry	5760	/cmm	4000-11000	K2EDTA WB
WBC Differential				
Neutrophils Method:Calculated	52.0	%	40-80	K2EDTA WB
Lymphocytes Method:Calculated	33.4	%	20-45	K2EDTA WB
Monocytes Method:Calculated	6.4	%	2-10	K2EDTA WB
Eosinophils Method:Calculated	**7.6**	%	1-6	K2EDTA WB
Basophils Method:Calculated	0.6	%	0-2	K2EDTA WB
Absolute Counts				
Absolute Neutrophils Method:Measured	2990	/cmm	1700-8800	K2EDTA WB
Absolute Lymphocytes Method:Measured	1810	/cmm	1100-4500	K2EDTA WB
Absolute Monocytes Method:Measured	370	/cmm	100-1000	K2EDTA WB

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Absolute Eosinophils Method:Measured	440	/cmm	20-500	K2EDTA WB
Absolute Basophils Method:Measured	30	/cmm	0-200	K2EDTA WB

Platelet Parameters

Platelet Count Method:Electrical Impedance/Flowcytometry	193000	/cmm	150000-400000	K2EDTA WB
MPV Method:Calculated	**11.4**	fl	6-11	K2EDTA WB

ERYTHROCYTE SEDIMENTATION RATE (ESR)

ESR Method:Photometric capillary stopped flow kinetic analysis	**11**	mm/hr	0-10	K2EDTA WB

CBC Comments:
A complete blood count (CBC) is a blood test which measures several cellular components of blood and is used to evaluate overall health and detect a wide range of disorders.
It is used to determine one's general health status; to screen for, diagnose, or monitor a variety of diseases and conditions that affect blood cells, such as anaemia, infection, inflammation, bleeding disorder or cancer.
A decrease in haemoglobin levels/ RBC count lower than the normal reference range for the given age can signify anaemia. Further investigation with iron studies, vitamin B12 and folic acid levels will help in determining the cause and assist in further treatment. Red cell indices help in classification of anaemia and give a clue to their etiology. An increase in haemoglobin level may signify polycythaemia, dehydration and is often seen in smokers.
White blood cell (WBC) counts and their differential counts (DC) are used to diagnose infections(bacterial/viral), allergies, inflammatory conditions and leukemias.
Platelets help in clotting of blood, any substantial decrease may increase the risk of bleeding

Note-
The differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood as per the recommendation of International council for Standardization in Hematology.

ESR Comments:
Erythrocyte sedimentation rate (ESR), is a blood test that detects non-specific inflammatory activity in the body. It is not a stand-alone diagnostic tool but can be used to help diagnose or monitor the progress of an inflammatory disease.
It helps to detect the presence of inflammation caused by one or more conditions such as infections, tumours or autoimmune diseases.
It is useful for diagnosing and monitoring of specific conditions such as temporal arteritis, systemic vasculitis, polymyalgia rheumatica, or rheumatoid arthritis where the levels are significantly increased.
Values are low in polycythaemia, hyperleukocytosis, hypofibrinogenemia and congestive cardiac failure and when there are abnormalities of the red cells such as poikilocytosis, spherocytosis, or sickle cells is present.
It is influenced by age, sex, stage of the menstrual cycle, pregnancy and medications taken (corticosteroids, contraceptive pills).

Note –
ESR is best interpreted when test is carried out on a fasting blood and needs to be clinically corelated.

Hema N Anadure

Verified By: 171886

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
TOTAL PROTEIN Method:Biuret	7.8	g/dL	5.7-8.2	Heparinized Plasma

Verified By: 173492

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
GLUCOSE, FASTING (F) Method:Hexokinase	90.0	mg/dl	Non-Diabetic: < 100 Prediabetes :100-125 Diabetic: >/= 126	Fluoride Plasma (F)

Comments:
Glucose is a simple sugar(monosaccharide) obtained by breakdown of carbohydrates. It is the main source of energy in the body. This test measures glucose levels in the blood after 8-12 hrs of fasting.

Clinical uses:
1. Diagnosis and monitoring of Diabetes(hyperglycemia)
2. Diagnosis and monitoring of Hypoglycemia
3. Diagnosis of some endocrinal disorders

High levels are seen in:
1. Diabetes – Type 1 and 2
2. Gestational Diabetes
3. Stress, surgery
4. Endocrinal disorders – Hyperthyroidism, Hypercortisolism
5. Pancreatic disorders – Pancreatitis, Glucagonoma
6. Drugs such as steroids, Hydrocortisone

Low levels are seen in:
1. Insulinoma
2. Overdose of Anti diabetic drugs & Insulin
3. Starvation
4. Endocrinal disorders – Hypothyroidism, Hypopituitarism, Addison's disease
5. Chronic kidney and liver disorders

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
GLUCOSE, POST PRANDIAL (PP) Method:Hexokinase	102.0	mg/dl	<140	Fluoride Plasma (P)

Comments:
Glucose is a simple sugar(monosaccharide) obtained by breakdown of carbohydrates. It is the main source of energy in the body. This test measures glucose levels in the blood 1.5-2hrs after a meal. In Non-diabetic individuals, levels peak at about an hour after the start of a meal, rarely exceed 140mg/dl, and return to pre-prandial levels within 2-3 hours. This normal time profile is altered in diabetics. American Diabetes Association do not recommend a PPG test for determining diabetes although it notes that postprandial hyperglycemia does contribute to elevated HbA1c levels and recommends testing and management of PPG levels for those patients who maintain optimum fasting blood glucose levels but have high A1C values.
FBS can occasionally be higher than PPBS in conditions like Early type 2 Diabetes, Exercise in between samples, Partial or total gastrectomy, Drugs like Oral Hypoglycemic agents, Salicylates, Betablockers, Pentamidine etc, Alcohol consumption or due to other conditions that cause rapid gastric emptying, excess insulin release and rapid glucose absorption.

Clinical uses: Diagnosis and monitoring of Diabetes(hyperglycemia), Hypoglycemia and some endocrinal disorders

High levels are seen in:

1. Diabetes – Type 1 and 2
2. Gestational Diabetes
3. Stress, surgery
4. Endocrinal disorders – Hyperthyroidism, Hypercortisolism
5. Pancreatic disorders – Pancreatitis, Glucagonoma
6. Drugs such as steroids, Hydrocortisone

Low levels are seen in:

1. Insulinoma
2. Overdose of Anti diabetic drugs & Insulin
3. Starvation
4. Endocrinal disorders – Hypothyroidism, Hypopituitarism, Addison's disease
5. Chronic kidney and liver disorders

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
SGOT(AST) Method:UV WITH P5P	**45**	U/L	15-37	Serum

Comments:
SGOT/AST is similar to alanine transaminase (ALT) in that both enzymes are associated with liver parenchymal cells. The difference is that ALT is found predominantly in the liver, with clinically negligible quantities found in the kidneys, heart, and skeletal muscle, while AST is found in the liver, heart (cardiac muscle), skeletal muscle, kidneys, brain, and red blood cells. As a result, ALT is a more specific indicator of liver inflammation than AST, as AST may be elevated also in diseases affecting other organs. If AST is more elevated than ALT, sources other than liver should also be considered.
AST is commonly measured clinically as a part of diagnostic liver function tests, to determine liver health.

High levels are seen in:
1.Viral hepatitis, liver damage, bile duct diseases
2.Diabetes,
3.Congestive heart failure,
4.Drugs such as zileuton, omega-3 acid ethyl esters, anti-inflammatory drugs, antibiotics, cholesterol medications, some antipsychotics such as risperidone, and anticonvulsants etc.
5.Infectious mononucleosis,
6.Myopathy
7.Reye syndrome

Low levels are seen in:
1.Uraemia,
2.Vitamin B6 deficiency
3.Drugs such as metronidazole, trifluoperazine.

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
SGPT (ALT) Method:UV WITH P5P	**81**	U/L	16-63	Serum

Comments:
ALT is similar to aspartate transaminase (AST) in that both enzymes are associated with liver parenchymal cells. ALT is found in plasma and in various body tissues but is most common in the liver. It catalyses the two parts of the alanine cycle. Serum ALT level, serum AST (aspartate transaminase) level, and their ratio (AST/ALT ratio) are commonly measured clinically as biomarkers for liver health.

High levels are seen in:
1.Viral hepatitis, liver damage, bile duct diseases
2.Diabetes,
3.congestive heart failure,
4.Drugs such as zileuton, omega-3 acid ethyl esters, anti-inflammatory drugs, antibiotics, cholesterol medications, some antipsychotics such as risperidone, and anticonvulsants etc.
5.infectious mononucleosis,
6.Reye syndrome

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
GGT Method:Modified IFCC	23	U/L	15-85	Serum

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
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TEST REPORT

Lab. Id	: **P2050512**	**Hosp. UHID**	:	**Reg. Date**	: 27-Apr-2024 / 12:02 PM
Name	: MR. MANAK SAHU .			**Collection**	: 27-Apr-2024 / 09:30 AM
Age/Gender	: 36Y-1D / Male			**Received**	: 27-Apr-2024 / 15:26 PM
Collected At	: ASTER CLINIC (MANYATA)			**Report**	: 27-Apr-2024 / 17:33 PM
Referral Dr	: DR WELLNESS.	**Report Status :** Final		**Print**	: 27-Apr-2024 / 18:41 PM
Bed	:				

Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
ALKALINE PHOSPHATASE Method:IFCC-PNPP With AMP Buffer	64.0	U/L	46-116	Serum

Comments:
ALP refers to a family of enzymes that catalyse hydrolysis of phosphate esters at an Alkaline pH. The day to day variation is 5-10%. Common drugs, including penicillin derivatives, drugs, antihistamines, cardiovascular drugs can increase blood levels.

Use:
Diagnosis and treatment of the liver, bone, intestinal and parathyroid diseases.

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

--- End Of Report ---

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
CREATININE Method:Enzymatic	**1.12**	mg/dl	0.60-1.10	Serum

INTERPRETATION:

PEDIATRIC REFERENCE RANGES

Reference range	mg/dL
0-1 year	0.04-0.33
2-5 years	0.04-0.45
6-9 years	0.20-0.52
10 years	0.22-0.59

Teitz 5th ed

COMMENTS: Results of creatinine tests are interpreted along with BUN results, and with other tests results that may have been performed at the same time, such as a renal panel. BUN and creatinine levels that are within the ranges established by the laboratory performing the test suggest that your kidneys are functioning as they should.

High levels can be seen in:
1. Damage to or swelling of blood vessels in the kidneys (glomerulonephritis) caused by, for example, infection or autoimmune diseases
2. Bacterial infection of the kidneys (pyelonephritis)
3. Death of cells in the kidneys' small tubes (acute tubular necrosis) caused by, for example, drugs or toxins
4. Conditions that can block the flow of urine in the urinary tract, such as prostate disease or kidney stone
5. Reduced blood flow to the kidney due to shock, dehydration, congestive heart failure, atherosclerosis, or complications of diabetes

Low blood levels can be seen in:
1. Lower muscle mass- muscular dystrophy, age related
2. Diet very low in protein

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
URIC ACID Method:Uricase	7.6	mg/dl	3.7-9.2	Serum

Note:- Serum Uric acid reference range has been updated with effect from 28/12/2022.

High levels (Hyperuricemia):

- Gout
- Kidney stones
- Chemotherapy
- Food items like dried beans, fishes like anchovies, mackerel
- Some herbal medications

Decreased in

- Drugs like high doses of Salicylates, probenecid, Cortisone, allopurinol
- Wilson Disease
- Fanconi syndrome
- Decreased in approximately 5% of hospitalized patients; most common causes are postoperative state (GI surgery, coronary artery bypass).

Limitations: A purine -rich diet (Liver, Kidney, Sweetbread) as well as severe exercise, increases Uric acid values

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
CALCIUM, SERUM				
CALCIUM Method:OCPC	9.3	mg/dl	8.5-10.1	Serum

Comments

Calcium is one of the most important mineral required by our body for a vareity of functions like bone development, muscle contraction, nerve signalling, blood clotting etc. Most of the calcium is stored in bones, while about 1% is available in circulation as either bound(to protein) or free forms(ionized).

High values are seen in:

1.Overactive parathyroid or thyroid gland
2.Cancer
3.Sarcoidosis
4.Tuberculosis
5.Long term Immobility
6.Drugs like thiazide diuretics

Low values are seen in:

1.Hypoproteinemia
2.Underactive parathyroid glands
3.Deficiency of Vit D, magnesium, dietary deficiency of calcium
4.Kidney failure
5.Pancreatitis

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
THYROID STIMULATING HORMONE				
THYROID STIMULATING HORMONE (TSH)	**7.45**	µIU/mL	0.55-4.78	Serum
Method:CLIA				

Comments:

Thyroid stimulating Hormone (TSH) is secreted by pituitary gland in response to thyroid hormone (T3 and T4) levels. It is secreted in larger amount if thyroid hormone levels are low and vice versa. TSH also exhibits circadian variations. Low levels are found during daytime and peaks are found just after midnight. The hypothalamic tripeptide hormone, Thyrotropin Releasing Hormone (TRH) directly stimulates TSH production.

The best way to avoid false fluctuations in lab test results is to have your thyroid levels checked under same conditions.

Clinical uses:
1.Screening for thyroid dysfunction and detecting mild (subclinical), as well as overt, primary hypo- or hyperthyroidism
2.Monitoring patients on thyroid replacement therapy, Prediction of TRH-stimulated TSH response
3.Confirmation of thyroid-stimulating hormone (TSH) suppression in thyroid cancer patients on thyroxine suppression therapy

Limitations:

1.This assay has not been validated for testing samples from newborns.
2.Heterophilic antibodies in human serum can react with reagent immunoglobulins, interfering with in vitro immunoassays.

High levels are seen in:
1. Primary untreated hypothyroidism
2. Hypothyroid patients on treatment, if receiving inadequate TSH replacement
3. Hashimoto's thyroiditis
4. Drugs such as Iodine containing agents, amiodarone, dopamine antagonists etc
5. Post subtotal thyroidectomy and Neck irradiation
6. Thyrotroph adenoma

Low levels are seen in:
1. Toxic multinodular goitre
2. Thyroiditis
3. Secondary pituitary or hypothalamic hypothyroidism
4. Drugs such as Dopamine agonists, levodopa, Antithyroid drugs etc

The American Thyroid Association (ATA) recommends screening for thyroid dysfunction beginning at the age of 35yr, especially in women, and every 5 years thereafter.

Verified By: 221344

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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MC-3362

TEST REPORT

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Age/Gender	: 36Y-1D / Male			**Received**	: 27-Apr-2024 / 15:01 PM
Collected At	: ASTER CLINIC (MANYATA)			**Report**	: 27-Apr-2024 / 16:07 PM
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Bed	:				

Clinical Pathology

Investigation	Result	Unit	Biological Ref. Interval	Specimen
URINE ROUTINE EXAMINATION				
PHYSICAL EXAMINATION				
Color Method:Manual	Pale yellow		Pale Yellow	Spot urine
pH-Urine Method:Methyl red sodium salt-bromthymol blue	6.0		4.5 - 8.0	Spot urine
Specific Gravity Urine Method:Bromthymol blue indicator	1.025		1.003-1.030	Spot urine
Transparency Method:Manual	Clear		Clear	Spot urine
Volume (ml) Method:Manual	30	ml		Spot urine
CHEMICAL EXAMINATION				
Proteins Method:Tetrabromophenol blue	Negative		Negative	Spot urine
Glucose Method:Oxidase-peroxidase	Negative		Negative	Spot urine
Ketones Method:Sodium nitroprusside	Negative		Negative	Spot urine
Blood Method:Tetramethylbenzidine	Negative		Negative	Spot urine
Bilirubin Method:Diazonium Salt	Negative	-	Negative	Spot urine
Urobilinogen Method:Diazonium Salt	Normal		Normal	Spot urine
Nitrite Method:P-arsanilic acid	Negative		Negative	Spot urine
MICROSCOPIC EXAMINATION				
Pus cells Method:Microscopy	04	/hpf	0-5	Spot urine
RBC Method:Microscopy	Absent	/hpf	0-2	Spot urine
Epithelial Cells Method:Microscopy	02	/hpf	0-5	Spot urine
Casts Method:Microscopy	Absent		Absent	Spot urine
Crystals Method:Microscopy	Absent		Absent	Spot urine
Amorphous debris Method:Microscopy	Absent		Absent	Spot urine

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MC-3362

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Bacteria Method:Microscopy	Absent	Absent	Spot urine

Interpretation

- A urinalysis is a group of physical, chemical, and microscopic tests done on a fresh urine sample. The tests detect and/or measure several substances in the urine, such as glucose, nitrites, bilirubin proteins, albumin, cells like RBC, WBC, casts, and bacteria.
- Many disorders may be detected in their early stages by identifying substances that are not normally present in the urine and/or by measuring abnormal levels of certain substances.
- As urine is produced by the kidneys, urine testing is useful in diagnosis and monitoring of diseases of kidney, urinary tract and bladder.
- Presence of significant amount of glucose in urine is an indication of diabetes.
- Presence of RBC casts, WBC casts or granular casts is indication of diseases of the Kidney.
- Presence of bacteria in urine should be correlated with clinical symptoms of urinary tract infection and urine culture reports.

Verified By: 168435

Dr. Apoorva P Gowda
MBBS MD (Pathology)
Specialist

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Biochemistry

Investigation	Observed Value	Unit	Biological Ref. Interval	Specimen
LIPID PROFILE				
TOTAL CHOLESTEROL Method:CHOD-POD	124.0	mg/dl	Optimal : < 200 Border Line : 200 - 239 High : >= 240	Serum
TRIGLYCERIDES Method:GOD-POD	174.0	mg/dl	Optimal : <150 Borderline-high: 150 - 199 High: 200 - 499 Very high: >500	Serum
HDL CHOLESTEROL Method:Elimination/Catalase	33.0	mg/dl	Desirable: >60 Borderline: 40 - 60 Low (High risk): <40	Serum
Non HDL CHOLESTEROL Method:Calculated	91	mg/dl	Optimal : <130 Above Optimal : 130 - 159 Border Line High : 160 - 189 High : 190 - 219 Very High : >=220	Serum
LDL CHOLESTEROL Method:Elimination/Catalase	71.0	mg/dl	Optimal: <100 Above Optimal: 100 - 129 Borderline High: 130 - 159 High: 160 - 189 Very High: >190	Serum
VLDL Method:Calculated	**34.8**	mg/dl	2.0-31.2	Serum
TOTAL CHOLESTEROL/HDLC RATIO Method:Calculated	3.76		0-4.5	Serum
LDLC/HDLC RATIO Method:Calculated	2.15		0-3.5	Serum

Comments:
Lipid profile is a panel of blood tests that serves as an initial screening tool for abnormalities in lipids, such as cholesterol and triglycerides. The lipid panel is used as part of a cardiac risk assessment to help determine your risk of heart disease and to help make decisions about what treatment may be best if you have borderline, intermediate or high risk.

The results of the lipid panel are considered along with other known risk factors of heart disease to develop a plan of treatment and follow up. Depending on the results and other risk factors, treatment options may involve lifestyle changes such as diet and exercise or medications that lower lipid levels, typically statins.

Additionally a lipid panel may be used to monitor whether treatment has been effective in lowering cholesterol levels.

NLA 2014-15 guidelines:
1. Non-HDL-C (calculated as total C – HDLC) represents the sum of cholesterol carried by all potentially atherogenic, apo B-containing lipoprotein particles, including LDL, IDL, Lp (a), VLDL (including VLDL remnants), and chylomicron particles and remnants.
2. An elevated level of cholesterol carried by circulating apolipoprotein (apo) B-containing lipoproteins (non- HDL-C and LDL-C, termed atherogenic cholesterol) is a root cause of atherosclerosis/ASCVD. HDL-C is responsible for lowering peripheral tissue cholesterol(reverse transport), inturn reducing risk of ASCVD.
3. Apolipoprotein B, hsCRP, Lp(a) and LP-PLA2 testing should be considered in patients with moderate risk for ASCVD.

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4. In all adults (>20 years of age), a fasting or nonfasting lipid profile should be obtained at least every 5 years. At a minimum, this should include total cholesterol and HDL-C, which allows calculation of non-HDL-C (total-C – HDL-C). If fasting (generally 9–12 hours), the LDL-C level may be calculated, provided that the triglyceride concentration is <400 mg/dL.
5. Apo B is considered an optional, secondary target for treatment. Epidemiologic studies have generally shown that both apo B and non-HDL-C are better predictors of ASCVD risk than LDL-C. Apo B and non-HDL-C share the advantage that neither requires fasting sample for accurate assessment.
6. Elevated triglyceride level is not a target of therapy per se, except when very high (>500 mg/dL). When triglycerides are between 200 and 499 mg/dL, the targets of therapy are non-HDL-C and LDL-C. When triglycerides are very high (>500 mg/dL, and especially if >1000 mg/dL), reduction to <500 mg/dL to prevent pancreatitis becomes the primary goal of therapy.
7. Lifestyle therapies for ASCVD risk reduction generally include interventions aimed at (1) dietary modifications (2) reducing total energy intake to lower body weight and adiposity for those who are overweight or obese; (3) exercise (4) improving risk factors associated with the metabolic syndrome (adiposity, dyslipidemia, high blood pressure, and elevated plasma glucose); and (5) ceasing tobacco use.

Verified By: 173492

Dr. Doddappa M B
PhD (Medical Biochemistry)
Consultant

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MER

Employee Name	MANAK SAHU				
Empoyee ID			Date	27/04/2024	
Age	36 yrs		Gender		
Name of centre			City		
Height (in mts)	175	BMI	26.3	Weight (in Kgs)	80.6
Waist circumference (in cms)		Hip circumference (in cms)		Waist-to-hip ratio	
Systolic BP	120	Diastolic BP	80		

Physician consultation

Gynaecology consultation (if any)

ASTER CLINIC
MANYATA TECH PARK
BANGALORE

EXERCISE STRESS TEST REPORT

Patient Name: MANAK SAHU,
Patient ID: 160060353
Height: 175 cm
Weight: 80.6 kg

DOB: 15.08.1987
Age: 36yrs
Gender: Male
Race: Indian

Study Date: 27.04.2024
Test Type: Treadmill Stress Test
Protocol: BRUCE
Medications:
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Medical History:
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Referring Physician: HC
Attending Physician: --
Technician: --

Reason for Exercise Test:

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Exercise Test Summary

Phase Name	Stage Name	Time in Stage	Speed (mph)	Grade (%)	HR (bpm)	BP (mmHg)	Comment
PRETEST	SUPINE	00:10	0.00	0.00	71	120/80	
EXERCISE	STAGE 1	02:34	1.70	10.00	106	120/80	
	STAGE 2	03:00	2.50	12.00	137	130/80	
	STAGE 3	01:43	3.40	14.00	166		
RECOVERY		05:03	0.00	0.00	100	130/80	

The patient exercised according to the BRUCE for 7:15 min:s, achieving a work level of Max. METS: 10.10. The resting heart rate of 72 bpm rose to a maximal heart rate of 166 bpm. This value represents 90 % of the maximal, age-predicted heart rate. The resting blood pressure of 120/80 mmHg , rose to a maximum blood pressure of 130/80 mmHg. The exercise test was stopped due to Target heart rate achieved.

Interpretation

Summary: Chest Pain: none.
Arrhythmias: none.
ST Changes: none.
Overall impression: Normal stress test.

Conclusions

*GOOD EFFORT TOLERANCE
NORMAL HR&BP RESPONSE
NO ANGINA/ARRHYTHMIAS
NO SIGNIFICANT ST -T CHANGES,
*STRESS TEST NEGATIVE FOR INDUCIBLE ISCHEMIA *

Physician_____ Technician_____ _____

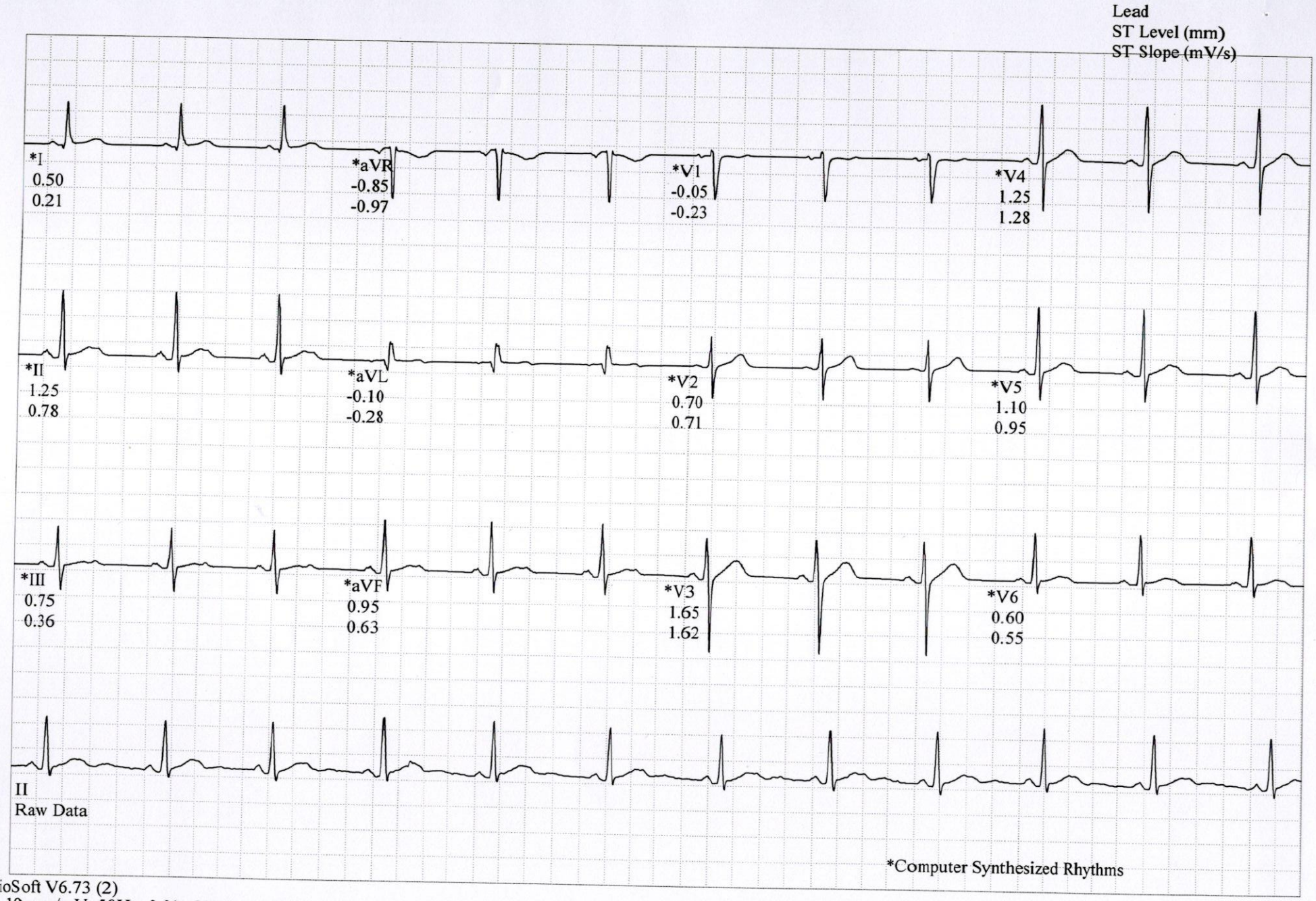
MANAK SAHU,
Patient ID 160060353
27.04.2024
11:25:36am

71 bpm
120/80 mmHg

Linked Medians
PRETEST
SUPINE
00:09

BRUCE
0.0 mph
0.0 %

ASTER CLINIC



GE CardioSoft V6.73 (2)
25 mm/s 10 mm/mV 50Hz 0.01 - 20Hz S+ HR(V4,V3)

Start of Test: 11:25:26am

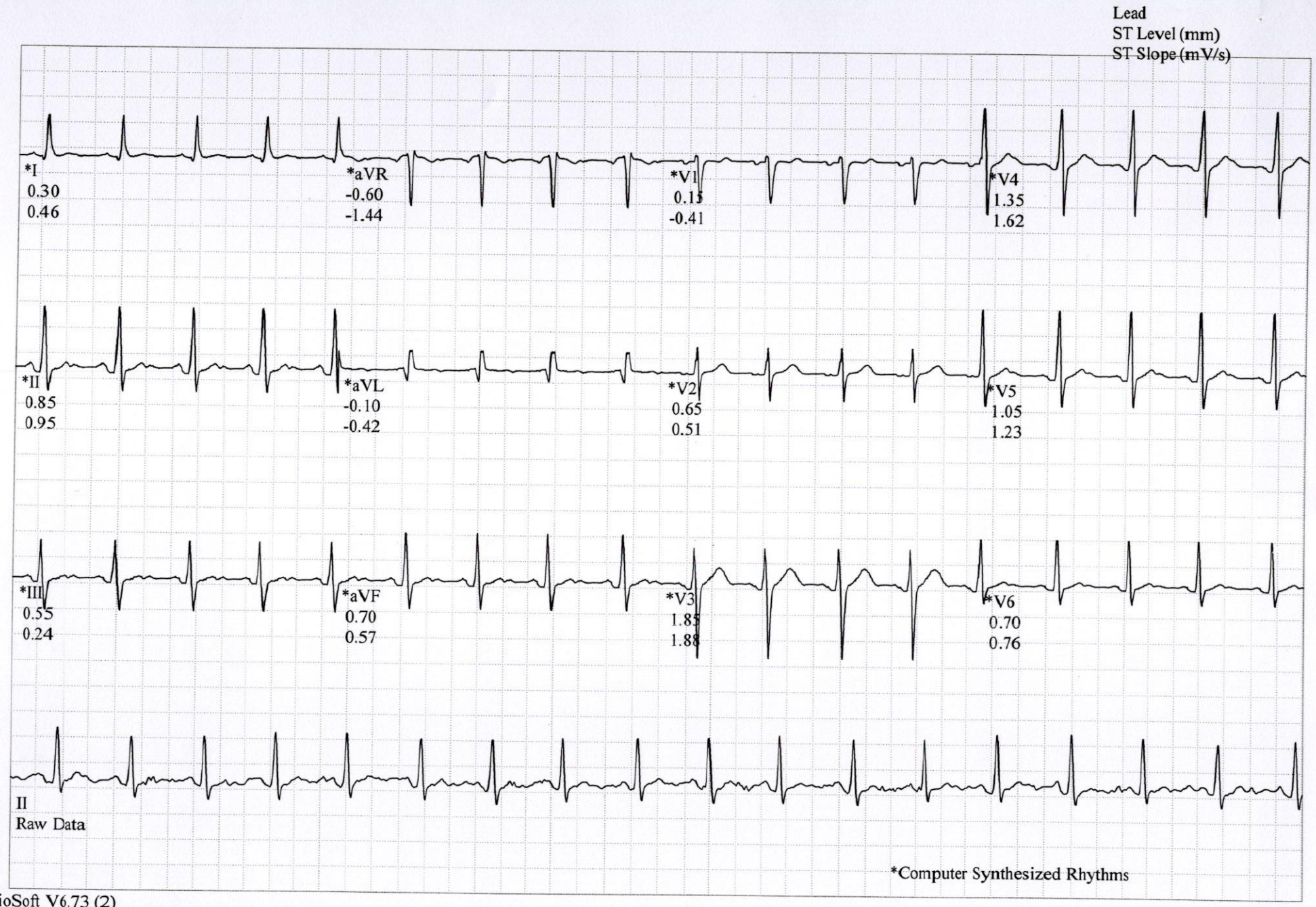
MANAK SAHU,
Patient ID 160060353
27.04.2024
11:28:08am

107 bpm
120/80 mmHg

Linked Medians
EXERCISE
STAGE 1
02:32

BRUCE
1.7 mph
10.0 %

ASTER CLINIC



GE CardioSoft V6.73 (2)
25 mm/s 10 mm/mV 50Hz 0.01 - 20Hz S+ HR(V4,V3)

Start of Test: 11:25:26am

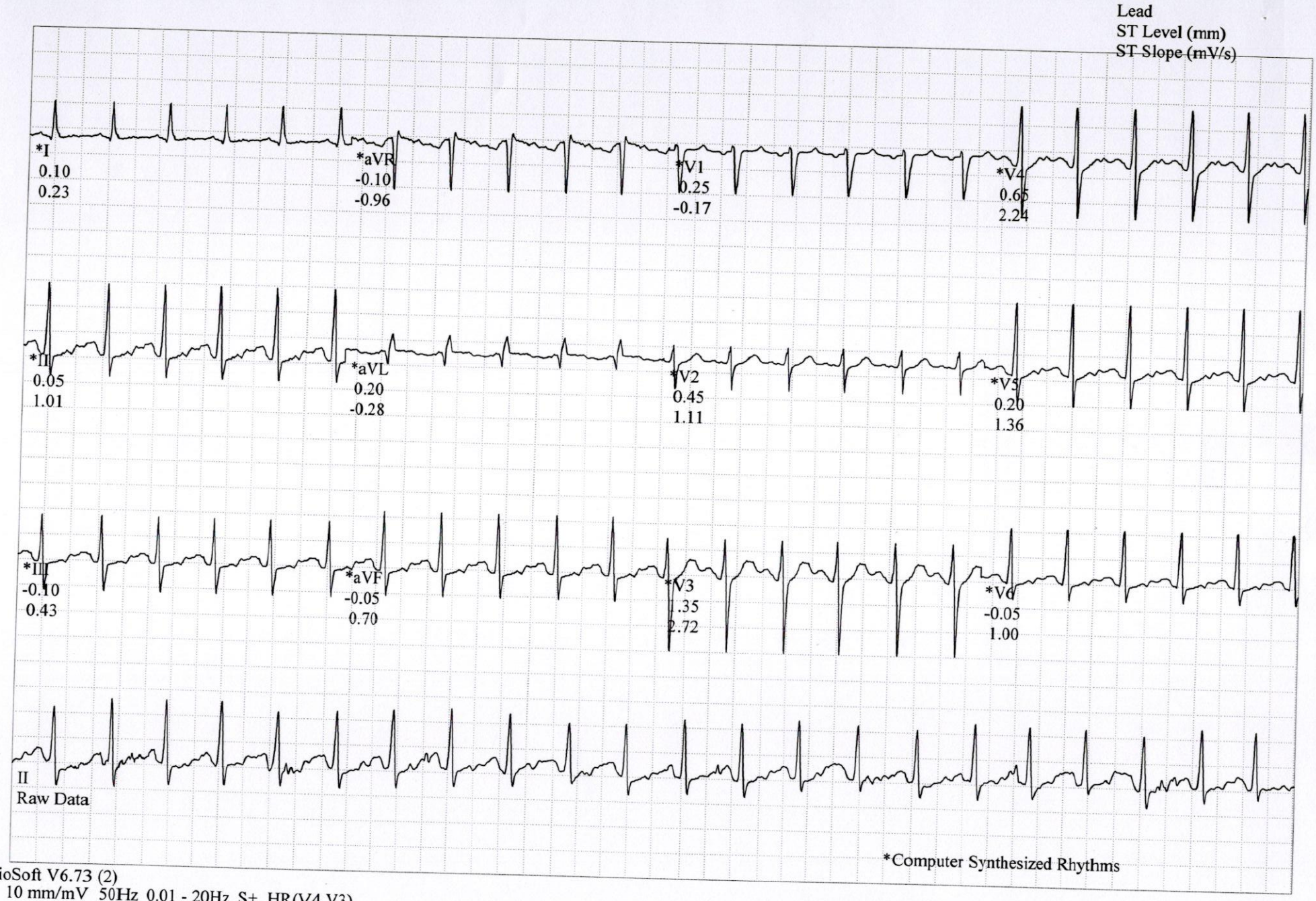
MANAK SAHU,
Patient ID 160060353
27.04.2024
11:30:58am

134 bpm
130/80 mmHg

Linked Medians
EXERCISE
STAGE 2
05:23

BRUCE
2.5 mph
12.0 %

ASTER CLINIC



GE CardioSoft V6.73 (2)
25 mm/s 10 mm/mV 50Hz 0.01 - 20Hz S+ HR(V4,V3)

Start of Test: 11:25:26am

MANAK SAHU,
Patient ID 160060353
27.04.2024
11:32:51am

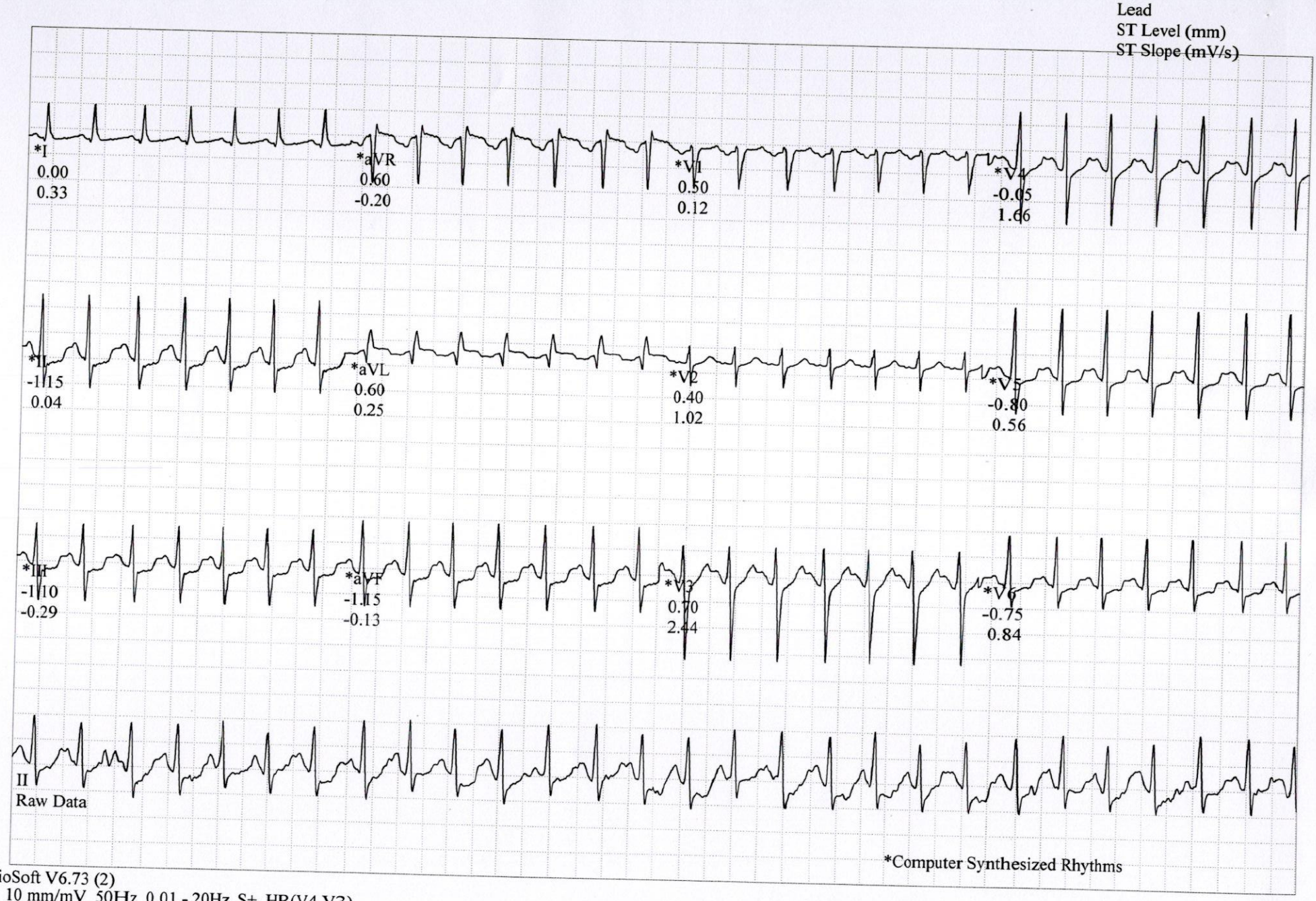
166 bpm

Linked Medians (PEAK EXERCISE)

EXERCISE
STAGE 3
07:16

BRUCE
3.4 mph
14.0 %

ASTER CLINIC



GE CardioSoft V6.73 (2)
25 mm/s 10 mm/mV 50Hz 0.01 - 20Hz S+ HR(V4,V3)

Start of Test: 11:25:26am

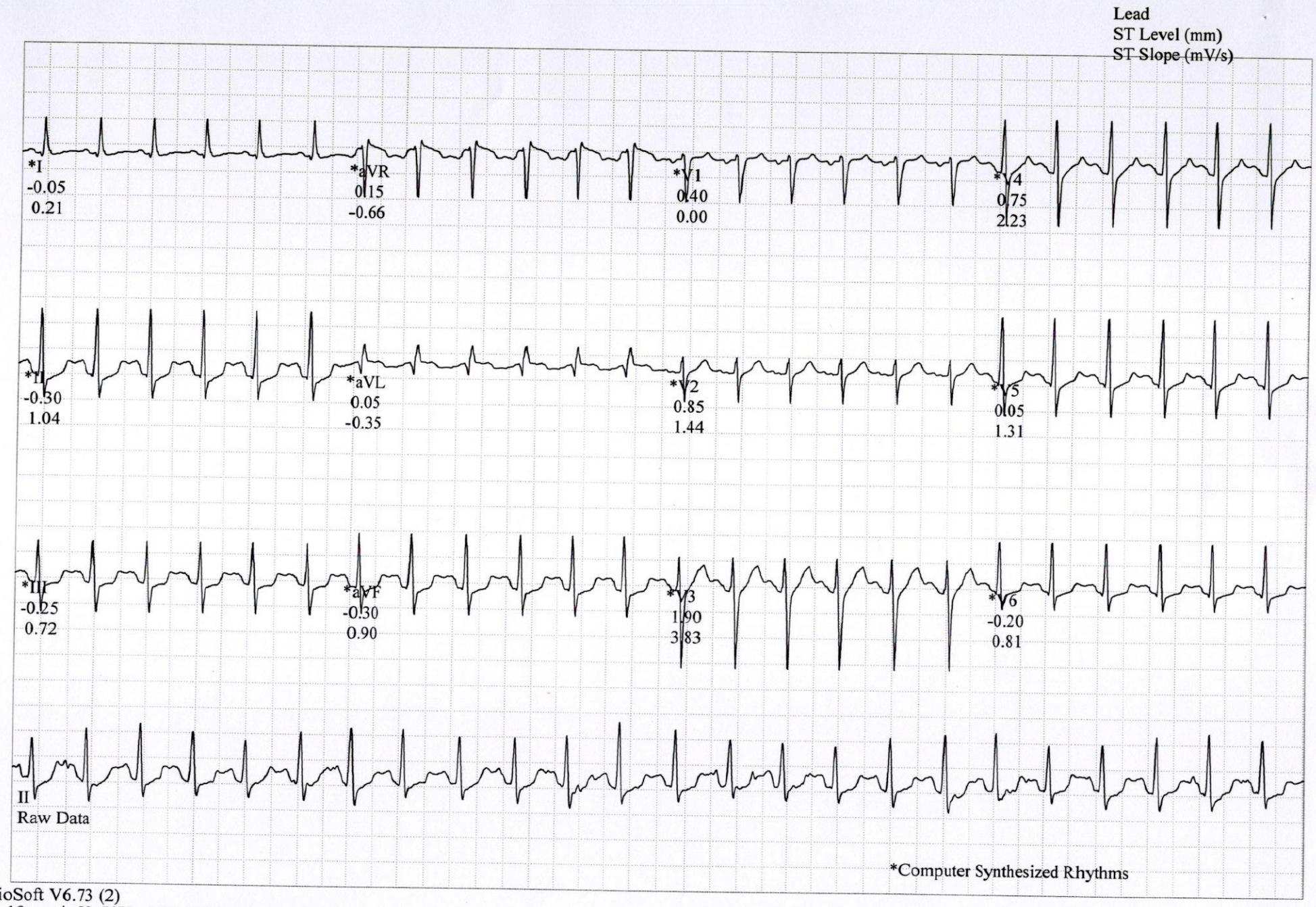
MANAK SAHU,
Patient ID 160060353
27.04.2024
11:33:50am

144 bpm

Linked Medians
RECOVERY
#1
01:00

BRUCE
1.5 mph
0.0 %

ASTER CLINIC



GE CardioSoft V6.73 (2)
25 mm/s 10 mm/mV 50Hz 0.01 - 20Hz S+ HR(V4,V3)

Start of Test: 11:25:26am

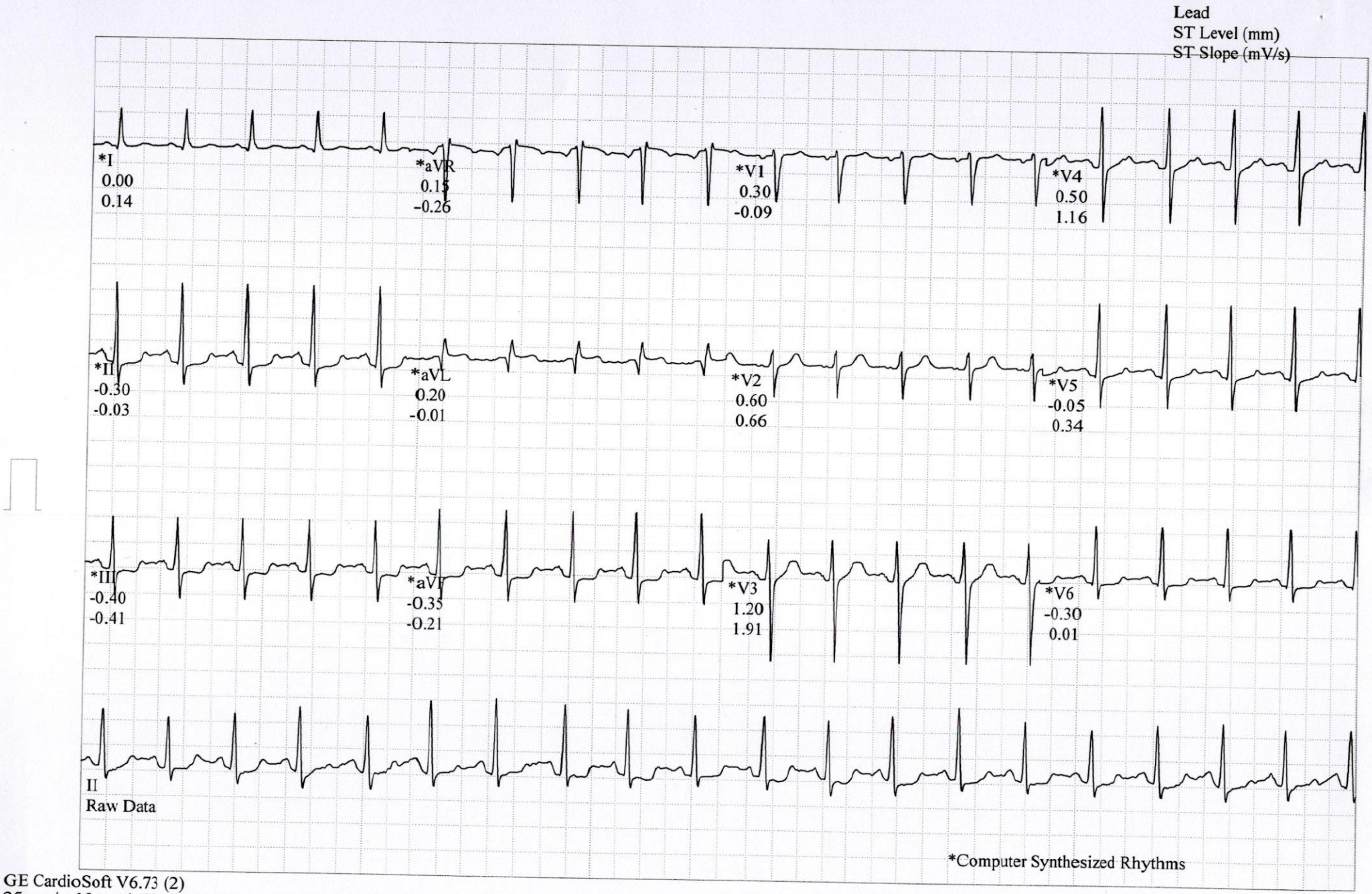
MANAK SAHU,
Patient ID 160060353
27.04.2024
11:35:50am

116 bpm
130/80 mmHg

Linked Medians
RECOVERY
#1
03:00

BRUCE
1.5 mph
0.0 %

ASTER CLINIC



GE CardioSoft V6.73 (2)
25 mm/s 10 mm/mV 50Hz 0.01 - 20Hz S+ HR(V4,V3)

Start of Test: 11:25:26am

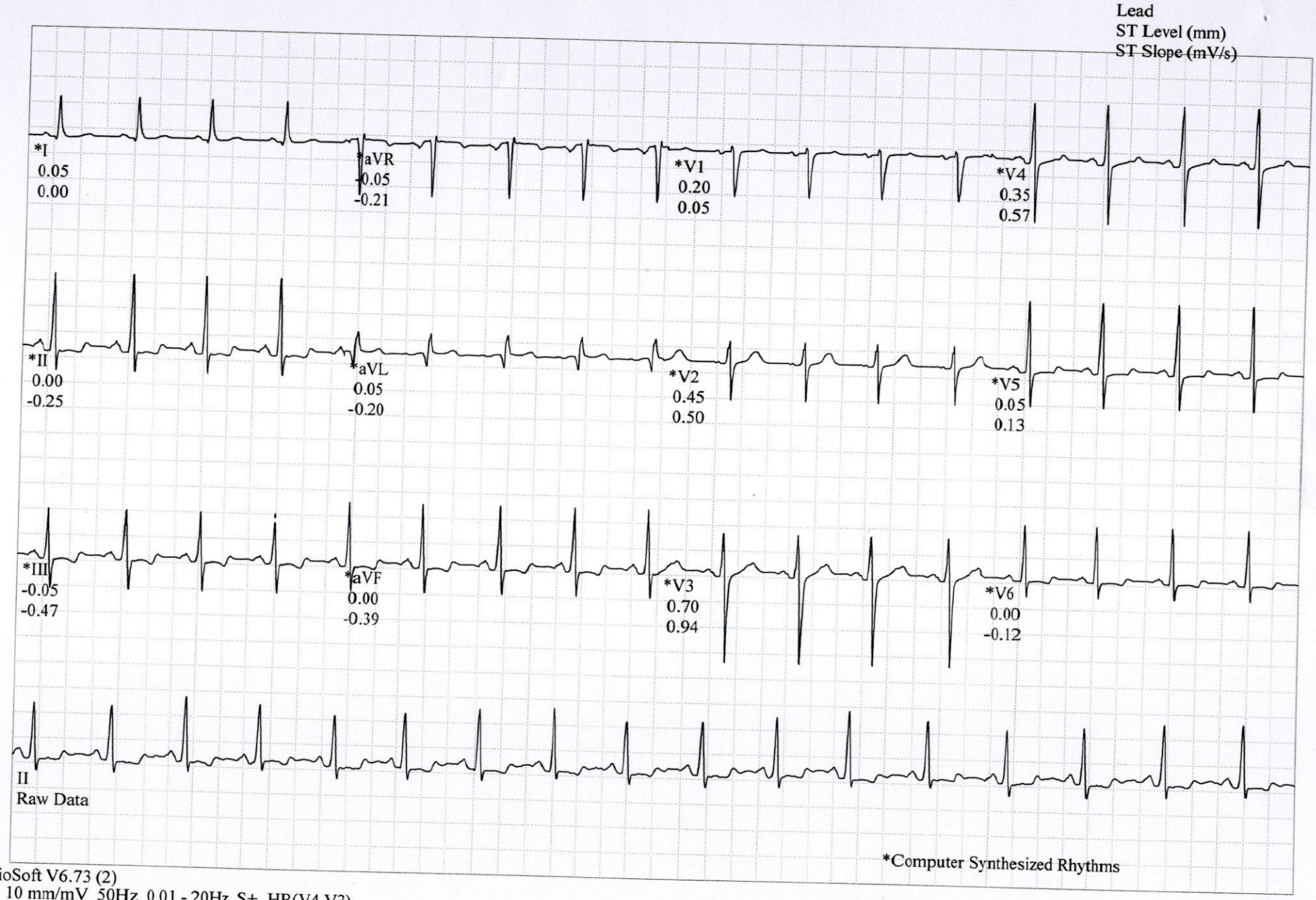
MANAK SAHU,
Patient ID 160060353
27.04.2024
11:37:50am

102 bpm

Linked Medians
RECOVERY
#1
05:00

BRUCE
0.0 mph
0.0 %

ASTER CLINIC



GE CardioSoft V6.73 (2)
25 mm/s 10 mm/mV 50Hz 0.01 - 20Hz S+ HR(V4,V3)

Start of Test: 11:25:26am

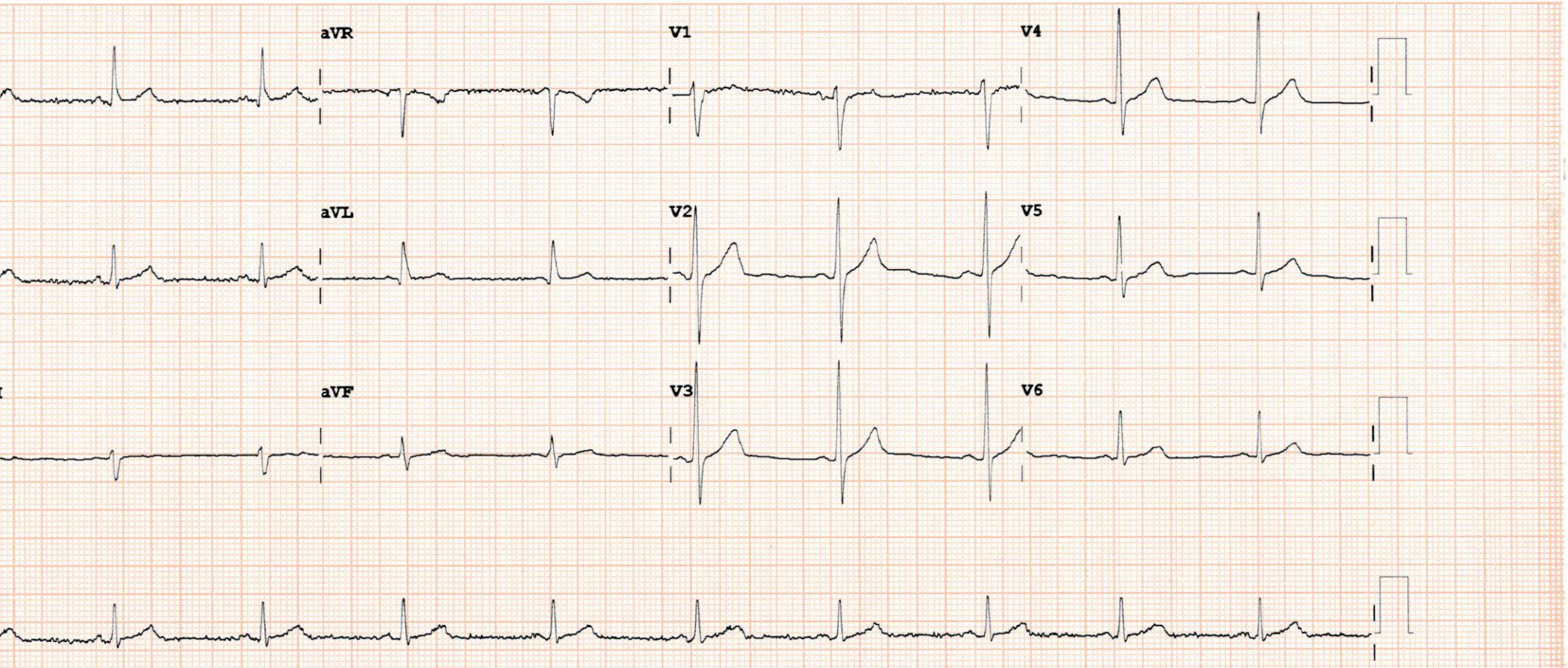
0353 MANAK SAHU 4/27/2024 9:56:45 AM
Years Male

te 59 . Sinus rhythm
. Abnormal R-wave progression, early transition
137 . Baseline wander in lead(s) V4
SD 89
377
c 374

AXIS--
12
s 10
36

- OTHERWISE NORMAL ECG -

Lead; Standard Placement Unconfirmed Diagnosis



vice: Speed: 25 mm/sec Limb: 10 mm/mV Chest: 10.0 mm/mV F 50~ 0.15-100 Hz PH100B CL P?

PHILIPS RECORDER M3708A

27/4/2024

Aster CLINIC

~~Mand~~
Manak
36 Y

NAME 11:48
27 APR 2024

VD= 0			
<R>	SPH	CYL	AX
	- 0.75	-1.75	85
<L>	SPH	CYL	AX
	- 0.25	-1.50	98

PD = 63
SHIN-NIPPON
ACCUREF K-900

Vision	RE	LE
	6/6	6/6
pupil	Brisk	Brisk

No colorblindnes

	SPH	CYL	AXIS	
RE	-	-1.75	85°	6/6
LE	-	-1.50	100°	6/6

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E : asterclinic.yelahanka@asterhospital.com

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TIN : 29441238072, Mfar Manyata Tech Park
Greenheart Phase IV, Nagavara Village,
Kasaba Hobli, Bangalore 560045, India.
T : 080 4655 8817 / 18 / 19 / 9513967880
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